



OFFICIAL

NMHS MHS PROCEDURE

Clinical Record Documentation

Scope (Audience)	All NMHS Mental Health Services (MHS) Staff
Scope (Area)	All NMHS MHS Areas
Summary	Procedure to ensure that all consumer and patient care service provision is accurately documented in a timely manner and are maintained according to Mental Health Act 2014 (MHA 2014), MP 0155/21 Department of Health State-wide Standardised Clinical Documentation for Mental Health Services Policy , current guidelines and standards.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Medication Safety (Std 4) Comprehensive Care (Std 5) Communicating for Safety (Std 6) Recognising and Responding to Acute Deterioration (Std 8)
Policy Sponsorship	Operation Co Director Community Adult MHS
Policy Contact	Manager Area Health Area Information, NMHS Mental Health
Version	7.0
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COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

North Metropolitan Health Service Mental Health Services (NMHS MHS) will ensure that all consumer and patient care activities are accurately documented in a timely manner and are maintained according to current guidelines and standards.

2. Background

An adequate clinical record must be maintained for each patient episode of care to:

- protect the legal and ethical interests of the patient, hospital and health care personnel;
- review and evaluate care given to a patient;
- afford a means of communication regarding episodes of care between health care professionals, including external agencies e.g. General Practitioner;
- enable effective continuing care of a patient at all points of admission, transfer and discharge; and
- support safety and quality in clinical care.

3. Risk

A failure to adhere to the following principles and guidelines may lead to compromised patient safety and non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☒

4. Procedure

All NMHS MHS are to ensure:

- Clinical record documentation meets the [Department of Health State-wide Standardised Clinical Documentation for Mental Health Services Policy](#), the [NMHS Clinical Documentation Policy](#), the [MP 0146/20 Department of Health Information Classification Policy](#) and the Australian Standard Healthcare Records Part 1: Paper Healthcare Records AS 2828.1:2019 and Part 2: Digitized Healthcare Records AS 2828.2:2019.
- Where relevant the [WA State-wide Standardised Clinical Documents \(SSCD\)](#) will be used as per the [Mental Health Policy Framework](#) and the [DoH Triage to Discharge Mental Health Framework for SSCD](#).
- Clinical Record documentation meets [National Safety & Quality Healthcare Service Standards 2nd edition](#).
- Clinical risks are documented and managed according to the [NMHS MHS Clinical Risk Assessment and Management \(CRAM\) Policy](#).

- Records are stored in a manner, which protects the environmental safety of the paper record and meets the requirements of the [MP 0178/23 Information Quality Policy – Resource Compendium](#), the [NMHS Healthcare Record Access, Use and Disclosure Policy](#) and the [MHS Clinical Record Management and Storage Procedure](#).
- Paper copy printouts of clinical information contained in computer software packages, e.g. PSOLIS, e-Discharge Summaries (other than NaCS summaries,) are to be filed in the designated section of the clinical record to ensure that a complete record is maintained.

PathWest laboratory reports will be recorded/accessed in the DMR via the iCM deeplink within the DMR. The iCM online order acknowledgement function should be used by medical staff to acknowledge PathWest laboratory results. All private laboratory results and radiology results should be wet signed by medical staff and placed in a filing tray for clerical staff to scan into the DMR.

- Compliance is regularly audited on an annual basis as part of routine safety, quality and performance monitoring.

This procedure is to be used in conjunction with the related policies, procedures and referenced documents listed at the end of this document. Staff should refer to the cited source documents for further elaboration or detail as required.

Where clinically relevant, the following minimum requirements will be included in the clinical record/DMR, in all programs within NMHS MHS. All MHS sites and services will have transitioned to the use of the Digital Medical Record (DMR) by June 2026. Refer to local processes for additional requirements at a service level.

4.1 Triage

Triage is the process of determining the urgency of response required for a consumer based on referral or presentation to mental health services. This process can be undertaken face to face or over the phone.

Purpose	Clinicians must triage all referrals. Triage documentation has been developed for use in both face-to-face and telephone triage but must include referrals from all sources. All referrals must be logged.
Target services	All mental health services conducting telephone or face-to-face triage.
Completion requirements	The triage modules can be completed by any appropriately qualified and experienced clinical mental health professional and must be completed in full.
Documentation title & format	Essential: Triage (electronic medical record– webPSOLIS) Downtime procedure (paper form): Triage document (SMHMR900)
Process	This occurs at the point of entry to any of the NMHS Mental Health Services. Best practice customer service to occur and should incorporate an assessment of need, risk complexity, acuity, advocacy and diversion to more appropriate services if indicated.

4.2 Initial Assessment

Assessment allows mental health clinicians to ensure complete, current and accurate documentation at each episode of care. Assessment documentation assists in identifying current and ongoing needs of consumers. During assessment and history documentation consent, legal status, Advance Health Directives and diversity needs can be identified.

Purpose	The assessment clinical document provides a framework for documenting a mental health assessment on first contact, or at other times when a comprehensive mental health assessment is indicated.
Target services	All mental health services conducting assessment
Completion requirements	The module can be completed by any appropriately qualified and experienced clinical mental health professional
Documentation title & format	Essential: MH Assessment Form (electronic – webPSOLIS) Downtime procedure (paper form): Mental Health Assessment (SMHMR902) and CAMHS001 for Child and Adolescents.
Process	The MH Assessment document is best completed once a consumer is admitted to a service. Based on the initial assessment, outcome measures and management plans can be completed and recorded on PSOLIS. Along with the MH Assessment, other evidenced based tools such as the Kessler 10 and Alcohol and other drug tools (DAST & AUDIT) if applicable, to inform the assessment. This assessment also includes the documentation of legal status, Advance Healthcare Directives, diversity needs, consent and carer/support person and General Practitioner (GP) contact details.

4.3 Risk Assessment

Mental health clinicians continually work towards improving all aspects of service delivery by identifying and adopting strategies to ensure risks are managed safely and effectively. Risk Assessments should include a structured and sensitive interview with the consumer and where appropriate carers or support persons. Risk assessment and management is informed by the policies and principles of the MH Clinical Risk Assessment and Management (CRAM) Policy. The Risk Assessment and Management Plan (RAMP) is the approved clinical document used to meet CRAM requirements.

Purpose	The aim of these documentation forms is to record information to help clinicians to make informed judgments about clinical assessment and risks, by providing a framework for good practice.
Target services	All mental health services conducting assessment
Completion requirements	The module can be completed by any appropriately qualified and experienced clinical mental health professional and be completed in full.
Documentation title & format	Essential: Risk Assessment and Management Plan (RAMP – webPSOLIS) Downtime procedure (paper form): Risk Assessment and Management Plan (RAMP) (SMHMR905) and (CAMHS002) for child and adolescent.
Process	The RAMP is to be completed at initial assessment and reviewed at a minimum every seven (7) days for inpatient acute services or every ninety (90) days for inpatient sub-acute and community services and additionally whenever there is a clinically significant change in presentation, circumstances or risk status. Where risk is identified or considered, strategies and actions, protective factors and mitigating approaches must be documented. Risk formulation and safety planning are mandatory components of the RAMP where any level of risk is identified.



4.4 Physical Assessment

All admissions to inpatient units will have a physical examination at the time of admission or within twelve (12) hours of admission. Community admissions should have a physical examination completed within one month of intake to a community team or evidence is available of a physical examination being completed at a preceding inpatient admission or having been completed within the past twelve (12) months by a GP.

Community activations should have a documented plan of care that has been discussed with the consumer's GP, noting the GP is current and has been provided an opportunity to participate in the care provision. This is essential where metabolic health is of concern or where medication prescribed by the team caused metabolic syndrome. Visiting GPs at health services such as Graylands Hospital could support this.

Purpose	The physical examination clinical document provides a structured format for the completion of a physical examination undertaken by a medical officer.
Target services	All mental health services. Where possible emergency departments and general practitioners.
Completion requirements	The clinical document can be completed by a medical officer or general practitioner. The initial component of the first page can be completed by an appropriately qualified and experienced nurse.
Documentation title & format	Essential: (web-PSOLIS) Mental Health Physical Examination Form (SMHMR903) and (CAMHS005) for child and adolescent.
Process	The Medical Officer (MO) or GP are to document if the consumer refuses to consent to the examination. If completed by a GP, correspondence must be filed, and findings documented in the Physical Assessment summary in the Initial Assessment documentation. In community, if a physical examination has been completed by the referring inpatient unit and is included in the care transfer documentation, this can be reviewed and filed as completed.

4.5 Physical Appearance

This document is used for medication safety and identification (in the absence of a wrist identification band or photo). This document supports existing protocols for identification verification particularly in community settings.

Purpose	The physical appearance clinical document provides a structured way of documenting a consumer's appearance. Physical appearance should be documented in the Appearance section of the mental state examination.
Target services	All mental health services providing assessment.
Completion requirements	The clinical document can be completed by any appropriately qualified and experienced clinical mental health professional and completed in full. This is to be completed at initial assessment with the service with the mental state examination documenting appearance.
Documentation title & format	Essential: Physical Appearance Form (SMHMR904), this is not applicable to Child and Adolescent consumers.
Process	Identify physical characteristics as noted on the form.

4.6 Care Planning

Care planning should be informed by the initial assessment, the consumers current and ongoing clinical needs, clinical judgement of the treating team or case manager, evidenced based outcome measures and the consumer and if appropriate carers and support persons.

Purpose	The treatment support and discharge plan provide a framework for summarizing the goals and clinical issues to be targeted in the episode of care, with the intent of aiding the monitoring of clinical status by the consumer, their carer or support persons, their GP and other service providers.
Target services	All mental health services providing treatment and intervention.
Completion requirements	The clinical document can be completed by any appropriately qualified and experienced clinical mental health professional. This must be completed in full.
Documentation title & format	Essential: Treatment, Support and Discharge Plan (electronic – webPSOLIS) <i>Optional</i> Crisis Awareness Plan CAPlan (electronic template in PSOLIS) <i>Optional:</i> Collaborative Action Plan CAPlan (brochure which can be used as an alternative to the above electronic CAPlan template in PSOLIS)
Process	After admission or activation, care planning should be commenced to identify the targets and choice interventions in collaboration with the consumer. If this cannot be done in collaboration, this will be documented, and the Treatment, Support and Discharge Plan (TSDP) completed focusing on current clinical needs. At each review of the TSDP collaboration with the consumer will be attempted. TSDP's will be reviewed as required and at the minimum every seven (7) (inpatient acute) or ninety (90) (inpatient sub-acute, community) days. The consumer will sign the initial TSDP and subsequent TSDPs. Where a consumer declines to sign the TSDP, it should be documented to note that it was offered and declined. TSDPs are to be completed and documented if the consumer wishes to complete. For Digital Medical Record (DMR) sites, any signed copy of the care plan is to be stored in the patients WIP file for later scanning into the patients DMR

4.7 Outcome Measures

The Health of the Nation Outcome Scales (HoNOS) usually referred to as:

- **"HoNOS"**;
- **"HoNOS65+"**; and
- **"HoNOSCA"** (for under 18-year-olds)

Consisting of a set of twelve (12) rating scales designed by the UK Royal College of Psychiatrists (RCP).

All versions of the HoNOS are available in PSOLIS with the default aligned to the program type e.g. Adult = HoNOS, Older Adult: HoNOS65+ and Child and Adolescent: HoNOSCA. The default can be changed where the consumer is 65 years or older or under the age of 18 years.

The Kessler10+ is the patient rated outcome measure, and the Life Skills Profile (LSP16) is completed for community services with the other measures.

Purpose	A multidimensional measure of problem severity capturing information across a broad spectrum of problem areas. It consists of 12 Items, rated in order from 1 to 12, with each item rated on a 5-point severity scale from 0-4, where 2 and above is a measure of clinical significance. Items 1 to 8 focus on the person. Item 1 addresses Overactive, aggressive, disruptive or agitated behaviour; Item 2, Non-accidental self-injury and suicidal ideation; and Item 3, Problem drinking or drug-taking. This initial placement of the three behavioural scales reflects the view that these issues may be of first concern in any acute presentation. Item 4 addresses Cognitive problems and Item 5, Physical illness or disability problems. This intermediate placement of these two items that address the person's cognitive and physical state, prior to items addressing psychiatric symptoms and social problems, reflects the view that the person's medical condition must be considered before contemplating their symptoms and social functioning. Item 6 addresses Problems associated with hallucinations and delusions; Item 7, Problems with depressed mood; Item 8 Other mental and behavioural problems. The remaining four Items 9 – 12 address social and role functioning together with the environmental factors that may support or inhibit the person's wellbeing and recovery. Item 9 addresses Problems with relationships; Item 10, Problems with activities of daily living; Item 11, Problems with living conditions; and Item 12, Problems with occupation and activities. Items 9 through 12 may be difficult to rate in an emergency admission to an acute inpatient service setting if information about performance is unavailable elsewhere. Nevertheless, the issues addressed by Items 11 and 12, although possibly difficult to rate, are likely to be significant in supporting the transfer of care, discharge planning, and in assisting the person's recovery once discharged. Phase of care: This is included in the HoNOS/HoNOS65+/HoNOSCA. A phase of care provides information describing the complexity of a consumer's presentation and the primary goal of care, which is reflected in the consumer's mental health treatment plan. Mental health phase of care is a prospective assessment of the primary goal of care at the time of collection, rather than a retrospective assessment. The mental health phase of care is independent of both the treatment setting and the designation of the treating service and does not reflect service unit type. An episode of care may have multiple mental health phases of care and the consumer's mental health care needs may change as they move between different phases of an episode and accordingly, the goal of care and the need for resources may change. The four mental health phases of care are: Acute: The primary goals of care are intended to reduce high levels of distress, manage complex symptoms, contain and reduce immediate risk. Functional gain: The primary goal of care is to improve personal, social or occupational functioning or promote psychosocial adaptation in a patient with impairment arising from a psychiatric disorder. Intensive extended: The primary goal of care is prevention or minimisation of further deterioration, and reduction of risk of harm in a patient who has a
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	stable pattern of severe symptoms, frequent relapses or severe inability to function independently and is judged to require care over an indefinite period. Consolidating gain: The primary goal of care is to maintain the level of functioning, or improving functioning during a period of recovery, minimise deterioration or prevent relapse where the patient has stabilised and functions relatively independently. Consolidating gain may also be known as maintenance.
Target services	All mental health services providing treatment and intervention.
Completion requirements	The clinical document can be completed by any appropriately qualified and experienced clinical mental health professional. A minimum of ten out of 12 questions must be completed. As each item in all versions of the HoNOS are weighted, full completion is preferred.
Documentation title & format	Electronic - JAVA PSOLIS
Process	Information should be gathered from: • the person; • direct observation; • other clinical staff in your service (nurses, clinical psychologists, etc.); • the person's psychiatrist or general practitioner; • the clinical record; • family and friends; and • any other clinicians or agencies involved in the person's care.

4.7.1 HONOSCA

A 15 item questionnaire, to be completed by a clinician, to indicate the severity of each problem, on a scale of 0-4. The measure is made up of two sections. The first section consists of 13 items relating to different types of problems; the second consists of two items relating the parent or young person's knowledge of the nature of the young person's difficulties and their information about the services available.

Purpose	All HoNOSCA items are answered on an item-specific anchored four-point scale with higher scores indicating more problems. A total score is calculated as the sum of the scores for items 1–13 only, with a range 0–52. Items scored 9 or with missing data are excluded from the calculation. The HONOSCA has 5 subscales: behaviour, impairment, symptoms, social and information.
Target services	All mental health services providing treatment and intervention.
Completion requirements	The clinical document can be completed by any appropriately qualified and experienced clinical mental health professional. A minimum of 13 out of 15 questions must be completed. As each item in all versions of the HoNOSCA are weighted, full completion is preferred.
Documentation title & format	Electronic - JAVA PSOLIS

Process	Information should be gathered from: • the person; • direct observation; • other clinical staff in your service (nurses, clinical psychologists, etc.); • the person's psychiatrist or general practitioner; • the clinical record; • family and friends; and • any other clinicians or agencies involved in the person's care.
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4.7.2 KESSLER10+

The Kessler Psychological Distress Scale (K10) is a psychological screening tool designed to identify adults with significant levels of psychological distress across the course of treatment. The K10 scale involves 10 questions about emotional states each with a five-level response scale. The measure can be used as a brief screen to identify levels of distress. The tool can be given to consumers to complete, or alternatively the questions can be read to the patient by the practitioner.

Purpose	The K10 is interpreted using a single total score of psychological distress, a broad concept characterised by unpleasant feelings or emotions that people may experience as overwhelming. The K10 can be used to identify more specific symptoms, including the capacity to delineate between Depression and Anxiety. The K10 is organised into four first-order factors and two second-order factors. The first-order factors are: • Negative Affect: Reflects emotional distress related to feelings of hopelessness, depression, and worthlessness, commonly observed in depression. • Nervous: Captures anxiety and nervousness, key indicators of anxiety disorders characterised by persistent worry and heightened tension. • Fatigue: Represents a lack of energy, reflecting the physical exhaustion often associated with depressive conditions. • Agitation: Involves restlessness and motor disturbances, which are strongly associated with anxiety. These first-order factors are grouped into two second-order factors: • Depression: Encompasses Negative Affect and Fatigue, covering both emotional and physical symptoms of depression. • Anxiety: Includes Nervous and Agitation, representing the core symptoms of anxiety seen in generalised anxiety disorder or among those experiencing a high degree of stress.
Target services	All mental health services providing treatment and intervention.
Completion requirements	Mandatory requirement.
Documentation title & format	Electronic - JAVA PSOLIS

Process	Provided to the consumer or completed with the case manager/inpatient staff member at admission, review and discharge alongside other outcome measures (HONOS – inpatient, HONOS and LSP16+ community) or when the goal of care changes (Phase of care).
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4.7.3 LSP16+

Purpose	The abbreviated Life Skills Profile (LSP-16) is a clinician-rated assessment that contains 16 items which provide a key measure of function to assess a consumer's abilities with respect to basic life skills over the last three months considering their age, social and cultural context. The focus of LSP-16 is on the person's general functioning, i.e. how the person functions in terms of their social relationships and ability to do day-to-day tasks. There are four subscales assessed within the LSP-16: Withdrawal: This subscale assesses an individual's tendency to withdraw from social interactions and activities. It may reflect the extent to which a person prefers solitude or faces challenges in engaging with others. (Lack of) Self-Care: The Self-Care subscale evaluates an individual's ability to independently manage personal care activities. It includes tasks such as grooming, dressing, and maintaining personal hygiene. (Lack of) Compliance: Compliance measures the degree to which an individual adheres to rules, instructions, or social norms. High scores on this subscale suggest less compliance and rule-abiding behaviour. Anti-Social : The Anti-Social subscale focuses on behaviours that are disruptive, defiant, or contrary to social norms. It assesses tendencies towards antisocial or oppositional conduct. Higher scores for the total score and subscale scores indicate a greater degree of disability. A total LSP scale score is calculated by adding individual scores for the whole scale together. Therefore, for the LSP-16, the total score can range from 0 to 48. As some items might not be relevant, items are optional and items with missing data are excluded from the calculation. An average score is provided for the total score and subscale scores so that comparisons can be made to determine relative areas of strength of challenges.
Target services	All mental health services providing treatment and intervention. The LSP is completed at a minimum, on activation, review and deactivation or when the phase of care has changed.
Completion requirements	Mandatory requirement.
Documentation title & format	Electronic - JAVA PSOLIS (Complete in PSOLIS then print copy for scanning into DMR).
Process	Process outlined in detail here: PSOLIS Clinical Staff Manual

4.8 Care Transfer Summary

Discharge planning is best commenced at admission in all mental health settings. This document reflects ongoing care that is needed on transfer to another government mental health service.

Purpose	Written communication of the current episode of care and its outcomes, and issues that require ongoing monitoring, when care is being transferred to another mental health service or other service provider.
Target services	All mental health services providing treatment and intervention.
Completion requirements	In inpatient services, the medical officer and in community settings, the community medical officer or appropriately qualified and experienced clinical mental health professional.
Documentation title & format	Electronic - webPSOLIS - Care Transfer Summary (SMHMR916) Essential for discharge or transfer of community patients from one clinic to another or to a GP. Not required for public mental health services that utilise alternative discharge summaries such as Notification and Clinical Summaries (NaCS). These need to be completed within 24 hours of patients discharge from a mental health unit.
Process	Any ongoing issues that require addressing in the receiving agency must be clearly identified. Contacts within the discharging agency and relative to the consumer must be identified. Inpatient services must have the MO sign the form. Community services must have the case manager sign the form. When being sent to receiving services, this form must be accompanied by the Treatment Support and Discharge Plan (TSDP) , Physical Examination Form or GP letter (s 4.4) , MH Assessment form and any other documentation relevant to the consumers' ongoing care including recording a final mental state examination, noting the consumer's mental state at the time of discharge

4.8.1 GP Letter

Purpose	Written letters to a general practitioner are at times undertaken to request and summarise care provision by a community mental health service. This section outlines the minimum requirements for written letters.
Target services	All mental health services providing treatment and intervention.
Completion requirements	If the progress notes refer the reader to a GP letter in lieu of a comprehensive clinical entry, the following parameters must be met: a) The date of the GP letter must be referred to and it is the responsibility of the Medical Officer to ensure that a copy of this letter is in the file within 24 hours of the clinical interaction The progress note entry in these instances must still contain at a minimum: comment of the mental state (e.g. if at baseline, or not); comment on current risk profile; comment on any actions or changes e.g. medication adjustments, when the next medical review will be, etc.
Documentation title & format	Electronic - webPSOLIS - Care Transfer Summary (SMHMR916)

	Essential for discharge or transfer of community patients from one clinic to another or to a GP. Not required for public mental health services that utilise alternative discharge summaries such as Notification and Clinical Summaries (NaCS). These need to be completed within 24 hours of patients discharge from a mental health unit.
Process	Any ongoing issues that require addressing in the receiving agency must be clearly identified. Contacts within the discharging agency and relative to the consumer must be identified. Inpatient services must have the MO sign the form. Community services must have the case manager sign the form. When being sent to receiving services, this form must be accompanied by the Treatment Support and Discharge Plan (TSDP), Physical Examination Form or GP letter (s 4.4), MH Assessment form and any other documentation relevant to the consumers' ongoing care including recording a final mental state examination, noting the consumer's mental state at the time of discharge

4.9 Progress Notes

Progress notes will succinctly refer to the progression of the treatment/care planning, including variations from the expected progress and changes to treatment plans including medications.

For non-DMR sites, progress Notes shall be integrated with Medical, Nursing and Allied Health professionals using the same record. For sites using the DMR, there are separate progress note templates for Medical, Allied Health and Nursing to use. These entries should be as contemporaneous as possible so that the DMR integrates all disciplines' progress notes into chronological order. Specific notation of change in risk or adverse events, indicating potential or actual harm to a patient or other, will be recorded, as a statement of fact. Evidence of ongoing information given to patient/family/carer/general practitioner/other care provider/s will be documented in the clinical record.

Every procedure and/or treatment intervention will be documented after completion. The documentation in the clinical records will record any complications related to the diagnoses and treatments, as a statement of fact. Post-procedure recovery instructions will be documented in the clinical records and signed.

Clinical discussions, liaison with external agencies, multidisciplinary team meetings and clinical reviews be documented in the progress notes. Progress notes and service events should document the content of all SMS entries with a consumer and the consumer replies, including any further actions taken by the clinician.

Documentation of clinical interactions should include relevant sections of the mental state examination (MSE) for example appearance, behaviour, speech, mood, thoughts, perceptions, cognition, and insight. Some interactions such as telephone appointments will not permit the completion of a full MSE. In these scenarios, clinicians are to document relevant observations available from the interaction, along with a clear plan for the next review, including timing, focus and any required follow-up actions within the progress notes.

The progress note entry in these instances must still contain at a minimum:

- comment of the mental state;
- comment on current risk;

- comment on any actions or changes e.g. medication adjustments, when the next medical review will be, etc.

4.10 Document Entry Requirements

All information documented in a patient's healthcare record must adhere to the following general principles:

- All documentation to be written in English.
- To be completed by one clinical mental health professional at one point in time.
- All documentation shall be in chronological order.
- Only authorised/approved forms, i.e. with a PMR Number, should be used to document patient information unless the documentation is within an electronic entry system only. (Refer to NMHS [MHS Medical Records Forms and Label Management Procedure](#)).
- Entries should be legible, concise, accurate, relevant, and contemporaneous manner, representing a factual account of observations and actions taken.
- Each page should be clearly identified with the patient's identification label. If receiving a record with a handwritten patient ID, the ID label is placed below the written name, not over it. On medical charts, the ID label is above the handwritten name. If labels are not available, then three identifiers must be handwritten into the Patient ID area. For DMR sites, every form MUST have a patient ID label (WebPAS "DMR set labels"), specific to the current episode of care. Spare Patient ID labels from previous episodes of care must be disposed of.
- Each entry shall include the following:
 - date;
 - time of entry and time of event within the body of the entry for Triage events and other significant events;
 - signature, plus printed surname in brackets; and
 - designation (e.g. Clinical Nurse Specialist (CNS), Registered Nurse (RN), Enrolled Nurse (EN), Social Worker, Occupational Therapist, Registered Medical Officer (RMO), Medical Student, etc.).
- Only approved symbols and abbreviations are to be used as per the [NMHS MHS Clinical Abbreviations Guideline](#).
- For non-DMR sites, no space is to be left between entries. If this is unavoidable, then a diagonal line must be drawn across the blank space to ensure additional entries cannot be made.
- Read previous entries to avoid repetition and to provide continuity.
- Record only facts and clinical impressions. Emotional statements, moral judgements and other non-factual statements must not be recorded.
- If additional information is added later, entries should be identified as "*Late Entry*" and time, date of the addition and signature documented.
- When documenting entries within the clinical record, do not spell out the names of other patients or staff who are not present or involved in care of patient unless necessary (initials are acceptable). Consider the purpose of the record and the fact that the information should be relevant to the care of the individual patient for whom the record is kept. Omission of other person's names also protects their privacy.

4.11 Document requirements specific to paper-based patient healthcare records

In addition to the general principles described above, documentation made in the paper-based patient healthcare record must adhere to the following requirements:

- All entries are to be made in **black ink pen only**. The only exception is Pharmacist notations on medication charts are in green for Mental Health and purple for other areas. Pharmacist entries into progress notes must be in black ink.
- No space is to be left between entries. If this is unavoidable, then a diagonal line must be drawn across the blank space to ensure additional entries cannot be made.
- Corrections should be made by ruling a neat line through the error, write “written in error” alongside, date and sign the incorrect entry.
- Correction fluid is not to be used.
- Cutting and pasting in paper format is not permitted.
- Printing entries onto PRM7 forms should be undertaken in reference to the principles outlined above, to ensure that existing entries are not over-written.

4.12 Document requirements specific to digital/electronic patient healthcare records

In addition to the general principles described above, documentation made in the digital/electronic patient healthcare record must adhere to the following requirements:

- An automatic digital signature is generated using the staff member’s ‘HE number’ and the name of the staff member logged onto the system. The digital signature is applied automatically and will record the time of the documentation entry. The staff member will record their designation where this is not automatically included in the system;
- Use of another staff member’s log in or identification to make an entry is not permitted (**Note:** staff supervising students may log in and supervise a student to make an entry in the healthcare record, the student must then ‘countersign’ by entering their name and designation).
- For DMR sites, when a correction of any errors is required, errors must be corrected by initiating a document “tweak” within the system (refer to DMR user documentation link for details of how to initiate a document tweak DMR Resources (health.wa.gov.au)). It is the responsibility of the person identifying the error to initiate the tweak. Errors include:
 - medical record forms which are filed in the wrong folder or against the wrong health service or specialty.
 - entries made against the wrong patient.
 - entries which are incorrect for any other reason.
- The clinical record is a legal document and may be used as evidence in court proceedings. Adherence to these guidelines will ensure that evidence taken from the clinical record is accessible, informative and minimises risk of adverse outcomes to patient, staff or health service.
- All clinical staff must use one integrated patient record for each patient and separate working files are not to be used.
- Documentation entries for **inpatient and community episodes of care must be entered contemporaneously** (wherever possible) on the same calendar day of each contact including face to face, telephone, text or email contacts. This is to be completed by the clinical staff member who has had contact with the patient or consumer. Late entries in the clinical record file will need to be dated and identified as late entries with all entries completed within 24 hours of the clinical interaction.

5. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance.

Training in site-specific documentation requirements within the clinical record should be provided at a local level. Professional Education and Training (PEaT) are to provide HoNOS and LSP training.

Evaluation will occur across the calendar year via multiple targeted audits as part of the Clinical audit schedule.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

6. Definitions

Terms	Definitions
Healthcare record	Includes a record of the patient's medical history, treatment notes, observations, correspondence, investigations, test results, photographs, prescription records and/or medication charts for an episode of care. This includes but is not limited to: - digital, electronic or paper-based records created in inpatient, outpatient and community settings; and - information collected via electronic systems (e.g. Digital Medical Record (DMR), webPAS, EDIS and i.Clinical Manager (iCM) PSOLIS, notation alerts)

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms.

7. Related Documents

Legislation	Mental Health Act 2014 (MHA 2014)
WA Health mandatory policies	MP 0155/21 Department of Health State-wide Standardised Clinical Documentation for Mental Health Services Policy Statewide Standardised Clinical Documentation for Mental Health Services Procedure Triage to Discharge SSCD MP 0178/23 Information Quality Policy Medication charts
NMHS Policy documents	NMHS Clinical Documentation Policy NMHS MH Clinical Risk Assessment and Management (CRAM) Policy NMHS Healthcare Record Management Policy
NMHS MHS Policy documents	Clinical Record Management and Storage Policy Patient Confidentiality Policy Release of Patient Information Policy
Standards	Chief Psychiatrist's Standards for Clinical Care Australian Commission on Safety and Quality in Health Care National Safety and Quality Health Service Standards

- [National Safety & Quality Healthcare Service Standards 2nd edition](#)

8. Resources

- Coombs, T., & Morris-Yates, A. (2025) Comprehensive guidelines for the reliable and valid rating of the Health of the Nation Outcome Scales (HoNOS) in Australian Mental Health Services. Sydney: Australian Mental Health Outcomes and Classification Network; Canberra: Australian Private Hospitals Association
[Australian mental health care classification mental health phase of care guide.pdf](#)
- [PSOLIS Clinical Staff Manual](#)

9. Document Control

Document Version Control			
Date	Version	Author	Notes
08/11/2018	5.0	A/Quality Improvement Coordinator	Scheduled Review: Complete re-work of policy to simplify and bring in line with current processes and enable easier understanding of staff requirements regarding documentation
16/01/2019	5.1	A/Policy and Audit Officer	Minor Amendment: updated DoH Hyperlinks.
08/02/2021	5.2	Policy and Audit Officer	Minor amendment: Link to rescinded DoH CRAM Policy removed from text and from References. Link to Mental Health CRAM Policy added. Link to superseding policy documents added to References. Updated hyperlinks.
08/08/2022	6.0	Director of Clinical Services	Scheduled review, additional overarching and related policies and references added, deletion of rescinded policies, information cross referenced with latest SSCD Triage to Discharge MH documentation. Amended from Policy to procedure, template and hyperlinks updated.
23/02/2023	6.1	Director of Clinical Services	Amendment to 4.7 Care Transfer Summary documentation, update to frequency of entries for inpatient and community episodes of care, hyperlinks updated.
11/02/2026	7.0	Manager, Area Health Information	Reviewed as per 3 yearly review cycle with amendment: Legislation, policy and references updated, DMR information added, section 4.7 outcome measures added, documentation requirements for services refreshed, compliance section updated, change in Sponsorship, content transferred to new template.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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